CHAPTER 8

From Freud to Prozac

Biological thinking gave psychiatry at the end of the twentieth century the capacity to be as science-driven as the rest of medicine. But this promise has remained unfulfilled, a result of psychiatry's enmeshment in popular values, in corporate culture, and in a boggy swamp of diagnostic scientism. While the psychiatry of the major mental illnesses took the high road of the neurosciences in the 1970s and after, the psychiatry of everyday affliction has tended to lose its way.

Science wanders astray easily in the world of quotidian anxiety and sadness, in the obsessive traits of behavior and the misfiring personality types that are the lot of humankind. Here the genetic trail grows dim and the neurotransmitters evaporate. Biology counts for little, culture and socialization for lots. Perhaps there are many separate disorders in this domain of neurotic illness, perhaps only a few, or none. The edges of this kind of psychiatry are poorly delineated, the boundaries between pathology and eccentricity vague. Despite its anchoring in the rest of medicine, psychiatry could easily drift aimlessly here.

It was not all psychiatry's fault. At the end of the twentieth century, a capital problem for psychiatry lay in a new tendency for people to psychologize distress, rather than to medicalize it as in the nervous diseases of yore, or to socialize it by putting it in terms of social class or gender. In the 1970s and after, personal issues that might once have brought people to the neurologist for counseling tended to become defined in psychological terms, meaning that people believe their difficulties capable of resolution through nonmedical psychotherapy. Problems for which nineteenth-century Americans might have sought out Weir Mitchell or George Beard tended in the new era to escape the medical gaze in order to meld into the mainstream of North American trouble-talk. Hence psychiatry, an arm of medicine, lost

out to nonmedical forms of counseling such as psychology and social work. As consciousness of the self in distress became part of society's general *Kulturgut*, the mission of psychiatry as the specialty of medicine best equipped to deal with such disorders became ever more threatened.

The pool of postmodern distress was enormous. By the early 1980s, the most recent period for which data are available, 22 million Americans a year were seeking help for mental problems.[1] Together, these individuals would make a third of a billion visits every year. By 1987, psychotherapy itself counted for 8 percent of all the nation's outpatient medical costs.[2] These are sizable figures. In a hundred years, psychotherapy had passed from an exotic procedure performed by neurologists to a virtual national pasttime: On a lifetime basis, more than a quarter of America's adult population would seek help from professional counselors of all descriptions.[3]

In this stunning rise in demand for psychological services, two developments were at work. One, the threshold of what people define as illness dropped sharply, increasing the overall volume of psychological complaints. Psychiatry itself had a clear role in lowering this threshold. Second, there was a tendency to seek therapy for nonillness, counseling for stress and for life's problems. This represented a psychologizing rather than a medicalizing of difficulties that previously might have been considered economic, social, or moral in nature and for which the help of the minister or a good neighbor might have been sought. These two events caused therapy for personal distress to flood way beyond psychiatry and out into the world of psychologists, psychiatric social workers, and other purveyors of what is termed generically "mental-health care." For psychiatrists whose income was based on office-practice psychotherapy, this outrush of potential business was alarming.

Maintaining Market Share

Reducing the threshold of what constitutes psychiatric illness was partially doctor-driven, partially patient-driven. Psychiatrists have an obvious self-interest in pathologizing human behavior and have been willing to draw the pathology line ever lower in their efforts to tear as much counseling as possible away from competing psychologists and social workers. Take, for

example, the question of boyhood. Whereas once Tom-Sawyer-esque enthusiasm was seen as part of the natural spirits of ladhood, in the 1960s and after, a whole series of psychological diagnoses arrived to define such behavior as pathological. The bidding opened high, with "minimal cerebral dysfunction,"[4] as it was called in the 1950s and 1960s. Tom Sawyer, in other words, had brain damage. When this diagnosis was discarded as obviously absurd, hyperactivity and inattentiveness were zeroed in on because boys can sometimes be exasperating to deal with in the classroom. Ignoring the perils of school-teacher psychiatry, educational professionals grasped gratefully for this new pathologizing of boyhood. In 1968, "hyperkinetic reaction of childhood (or adolescence)" entered the official nomenclature, supposedly manifest in restlessness and distractibility.[5] In 1980, this became officially known as "attention deficit disorder with hyperactivity." [6] It is still unclear whether there is some core group of those diagnosed as "ADD" who have a real organic disorder. The point, however, is that medical therapy for it could be done only by MDs, prescribing an amphetamine-like compound called "Ritalin" (methylphenidate). By 1995 doctors were writing 6 million prescriptions for Ritalin a year, and 2.5 million American children were on the drug.[7] This is one way of maintaining market share.

Since ancient times, both boys and girls have become anxious about scary stories. Yet it would have occurred to no one across the centuries to give psychiatric diagnoses to these anxieties about fantasms, not at least until the advent of "posttraumatic stress disorder" (PTSD), a syndrome initially associated with the trauma of combat (see pp. 304-305). Whether a distinctive veterans' psychiatric syndrome involving stress actually exists is unclear. But even if it exists, once PTSD became inserted into official psychiatric lingo, the popular culture grabbed it and hopelessly trivialized it as a way of psychologizing life experiences. By 1995, therapists were talking about "PTSD" in children exposed to movies like *Batman*. According to one authority, 80 percent of children who had watched media coverage of a crime hundreds of miles distant exhibited symptoms of "posttraumatic stress."[8] The anxieties of the children themselves were nothing new under the sun. New was psychiatry's willingness to persuade parents that the quotidian problems of maturation represent a distinct medical disorder.

The boundaries of what constitutes depression have been expanded relentlessly outward. Depression as a major psychiatric illness involving bleakness of mood, self-loathing, an inability to experience pleasure, and

suicidal thoughts has been familiar for many centuries. The illness has a heavy biological component. Depression in the vocabulary of post-1960s American psychiatry has become tantamount to dysphoria, meaning unhappiness, in combination with loss of appetite and difficulty sleeping. Thus it comes as no surprise that the incidence of depression so defined has been rising steadily and occurring at ever younger ages.[9] In 1991, the National Institute of Mental Health began organizing a "National Depression Screening Day," in the context of its "Mental Illness Awareness Week." Such programs encourage family doctors to diagnose depression more often in their patients and refer them to psychiatrists. Although this is partly legitimate—a missed major depression may result in a patient's suicide—the ultimate effect is psychiatric empire-building against other kinds of care. Indeed, the American Psychiatric Association jubilates over "record numbers" each year.[10] As a consequence of this continual hammering of the depression theme, depression has become the single commonest disorder seen in psychiatric practice, accounting for 28 percent of all patient visits.[11] (The availability of drugs such as "Prozac," said to be specific for an entire "spectrum" of affective disorders [see pp. 323-324], has doubtless contributed as well to increasing the diagnosis of depression: Physicians prefer to diagnose conditions they can treat rather than those they can't.)

Personality disorders have become a whole sandbox for empire building. Although the concept of a disorder of the personality—in which everybody suffers but the patient—remains scientifically rather murky, in practice imputed personality disorders have taken off. Diagnoses such as antisocial personality disorder arose preferentially in private psychiatric practice and were virtually unknown in other medical settings.[12] Multiple personality disorder (MPD) roared in from obscurity to become epidemic in the 1980s.[13] Other so-called disorders of personality represented merely the exaggeration of familiar character traits. Yet the entire notion of giving patient-status to people because they are troublesome to others represented a pathologizing of essentially normal if irksome behavior. Thus these diagnoses of personality, as well as the other ballooned disease labels, dipped greatly the threshold at which individuals were said to be ill.

Lowering the threshold increased the patient base in a historically unprecedented manner. If in the nineteenth century, psychiatry had focused on psychotic inpatients, and in the early twentieth century on neurotic outpatients, at the end of the twentieth century psychiatry was increasingly

interested in the kinds of patients previously seen by family doctors or not seen medically at all. Leaders in the field started speaking of “minor depression, mixed anxiety-depression, and mild neurocognitive disorder as ... conditions that may deserve consideration as separate categories.” The notion of “subthreshold symptoms” gained currency as a means of reaching out to “previously subthreshold patients.”[14] This is the language of empire-building and market-conquest. The evidence that these conditions represent diseases, or disorders, in the sense that mumps and major depression are disorders, is extremely slim.

In insider discussions, psychiatrists were perfectly frank with one another about shifting the focus from disease to unhappiness. Said Robert Wallerstein, a senior figure in the profession, in 1991:

> Much of what we treat people for in intensive individual psychotherapy—gross dissatisfactions with the course of their life, difficulties in the area of interpersonal relationships or work adjustment ... school or work inhibitions, and so on—are not considered by the third-party payers [insurance companies] to be formal diseases for which they should be expected to carry the treatment costs. After all, the inability of a graduate student to complete a doctoral thesis in comparative languages ... is hardly considered a disease state for which an insurance program should provide treatment.

Wallerstein emphasized that the student might be disappointed and frustrated, and deserving of help.[15] Yet his problems were not psychiatric in the classic sense, rather of the subthreshold variety.

In many ways, therefore, psychiatry was pushing the envelope. In their struggle to maintain themselves against nonmedical competition, psychiatrists found irresistible the temptation to take familiar, real illnesses and expand their margins. This was not unlike the efforts of nineteenth-century neurologists to pathologize various internal sensations in women with the “hysteria” diagnosis and make hysteria virtually coterminous with femininity.[16] But now it was depression and the like that were being pumped up to give them uncommon currency. Memphis psychiatrist Hagop Akiskal slaked “the dilution of the concept of mental illness to such a degree that it is no longer meaningful.” He cited “a pseudoepidemic of mental illness because of fashionable diagnoses like borderline schizophrenia.”[17] In the continuum of symptoms that runs from major depression to sadness, or from

schizophrenia to eccentricity, psychiatry pushed the boundary of pathology steadily to the right, away from the unwell and toward the commonplace. The message was that these ubiquitous features of the human condition represented billable psychiatric illnesses.

A Nation Hungers for Psychotherapy

Underlying this expansion of the envelope was psychiatry's determination not to be left behind in the public's rising clamor for psychotherapy. As psychiatry attempted to maintain its own market share, competition in the psychotherapy marketplace was booming. The earliest challenge had come from social work, which discovered psychotherapy in 1904 when Adolf Meyer's wife Mary Potter Brooks, who is considered the first American social worker, began visiting the families of patients on Ward's Island where Meyer was superintendent.[18] After World War I, social-work leaders began to shift the profession systematically from the provision of social services to psychotherapy. In 1920, at the National Conference of Social Work in New Orleans, Philadelphia's Jessie Taft termed social work "fundamentally psychological, or, if you please, psychiatric" in nature. And Mary Richmond, director of the charity division of the Russell Sage Foundation in New York, called for "social treatment" of families from the viewpoint of "small-group psychology."[19] The number of psychiatric social workers rose from around 2,000 in 1945 to almost 55,000 in 1985 (compared with 32,000 psychiatrists in 1985).[20] By 1990, there were 80,000 "clinical" social workers in the United States.[21] In that year a quarter of the members of the National Association of Social Work were in part- or full-time private practice.[22] The elective terrain of these social workers, in the words of Harry Specht, a senior professor of social work at Berkeley, was "'the worried well,' white middle class, 20-40-year-olds who are unhappy, unfulfilled, and unsatisfied."[23] This was exactly the client base of post-1970s psychiatry.

The psychologists, late in repositioning themselves from testing to psychotherapy, also offered massive competition. It was only with the publication of psychologist Carl Rogers' book Client-Centered Therapy in 1951 that psychologists jumped into the psychotherapy market. The

“humanistically oriented psychotherapy” of Rogers did not require mastering some complicated therapeutic system like Freudian or Jungian analysis but rather feel-good beliefs about reconstructing the self. Rogers shunned systematic “methods” of psychotherapy. Being client-centered meant having “the faith or belief in the capacity of the individual to deal with his psychological situation and with himself.”[24] It was, said Rogers, the idea “that the individual has within himself vast resources for self-understanding, for altering his self-concept, his attitudes, and his self-directed behavior—and that these resources can be tapped if only a definable climate of facilitative psychological attitudes can be provided,” namely psychotherapy of the kind conducted by psychologists. “[My method] turned the field of counseling upside down,” said Rogers later.[25] In the world of psychotherapy, his ideas became distilled as, in the words of a somewhat bilious Specht, “You can do it; you’re wonderful and good, there is richness in you, if it feels good on you it’s okay.”[26]

Initially, the psychiatrists reacted sharply to the inroads of the psychologists, claiming they were practicing medicine without a license. At the University of Rochester, the psychiatry department tried to put Rogers’ Guidance Center out of business. After Rogers moved to Chicago, only a sustained campaign on his part prevented the psychiatry department of the University of Chicago from closing his fledgling Counseling Center.[27] The psychologists finally prevailed. By 1996, the clinical psychology branch of the American Psychological Association had almost 7,000 members, to say nothing of other psychologists also offering psychotherapy. [28] By the 1990s, clinical psychologists and social workers together vastly outnumbered psychiatrists. “Psychotherapy ... has become a predominantly nonmedical activity,” mourned the Group for the Advancement of Psychiatry.[29]

Who was demanding the services of all these new mental-health professionals? Was it individuals with psychiatric illnesses, however broadly defined? No. To be sure, there was in American society a great volume of illness. By the estimate of the National Comorbidity Survey (NCS), a random sample of the American population conducted in the early 1990s, 48 percent of adult Americans would have a psychiatric or substance-abuse disorder over the course of the life cycle, and 30 percent had experienced such a disorder within the previous year of the survey. (The survey defined psychiatric in the widest possible terms, pushing the envelope truly to its outer limits.) These statistics, if accurate, made psychiatric problems among

the commonest medical problems in American society, roughly of the same order of frequency as high blood pressure.[30]

Yet these individuals with psychiatric problems were not the main clients for psychotherapy. The NCS study found that only 21 percent of people with disorders were being treated.[31] The other four-fifths had no interest in psychotherapy or treatment and evidently were willing to coexist, happily or unhappily, with their problems.

Instead, the great demand for psychotherapy was coming from people unhappy with their lives but who were not experiencing the depression, anxiety, obsessive-compulsive behavior, and other symptoms characteristic of psychiatric illness. Various studies showed significant numbers of individuals in psychotherapy who had no psychiatric problems. For example, a survey of the Canadian province of Ontario conducted in 1990 found that "forty-two percent of those who have sought help in the past year apparently did not have a mental disorder during that time."[32] As Daniel Freedman, an influential biological psychiatrist at UCLA, put it, "Everyone has problems. But not everyone has symptoms, and fewer have a disorder."[33]

In the 1990s, psychiatry was being bent out of shape by a colossal kind of failure-to-fit. Psychiatrists had been trained for one thing and ended up treating another. They had trained as residents to treat the major psychiatric illnesses. But once in office practice, they gravitated to the commoner and more lucrative psychoneuroses. In doing so, they found themselves in direct competition with the social workers and psychologists. Rather than returning to the main psychiatric diseases, the terrain of choice of biological psychiatry, they went in the opposite direction, expanding the definition of illness to include behavior and symptoms previously reckoned as "subthreshold," and catering to the great American public's demand for psychotherapy in dealing with problems of living.

Even in terms of economic self-interest, remaining on the high road of science would have been preferable, for it was mastery of the neurosciences that distinguished psychiatry from its competitors. And the embarrassing jostling with nonmedical therapists for business could not help but drag down the reputation of psychiatry in the eyes of the public. In 1995, for example, the American Psychiatric Association lobbied the Congress fiercely to keep psychologists from prescribing drugs, humiliating psychiatry in full public view for its recourse to self-interest rather than to science.[34] Such episodes created the impression that psychiatrists were distancing themselves ever

further from their scientific claims to be a medical specialty. In terms of diagnosis and therapy, therefore, American psychiatry had begun to lose its way.

Science versus Fashion in Diagnosis

The risk of wandering in the wilderness heightened as psychiatry took on the challenge of diagnosis and found it as much driven by politics as science. Diagnosis in medicine is important because it gives the key to therapy and prognosis, matters of vital significance to the patient. Compared with psychiatry, diagnosis in other medical specialties is relatively straightforward because the causes of most conditions are known: In pathology class, the medical students memorize the causes of disease in an organ system as: "traumatic, infectious, neoplastic, toxic, autoimmune ..." and so on. The causes of most organic diseases are relatively limited in number and not a matter of great controversy. Just imagine a group of respirologists having heated battles over the cause of pneumonia, splitting into separate societies and founding their own journals over the issue. Not so in psychiatry where, genetics apart, the causes of few conditions are known. What is the cause of something like erotomania, the delusional belief that someone else is in love with you? Nobody knows. Psychiatric illness has tended therefore to be classified on the basis of symptoms rather than causes, which is where the rest of medicine was in the nineteenth century. Grouping the various symptoms into larger disease categories can thus be somewhat arbitrary: Is erotomania part of schizophrenia, part of delusional disorder, or a separate illness? And the groupings themselves have been historically highly controversial, with entire schools of psychiatry rising and falling over the issue of how to classify "hysteria." These considerations suggest that in classification it is very easy for psychiatry to lose its way.

Adding to the risk is the existence of various national traditions, so that a given patient might be diagnosed quite differently from one country to another. One thinks of "eruptive delusional disorder" (*la bouffee delirante)* in France, a diagnosis that has no counterpart elsewhere. "The English call almost any kind of emotional trouble 'neurosis,'" said Henri Ellenberger, the great historian of psychiatry, in the mid-1950s. "The French apply the

diagnosis of feeblemindedness very liberally." As for the Swiss, "The French say that the Swiss diagnose schizophrenia in '90 percent of the psychotics and 50 percent of the normal.'"[35]

But nobody used the diagnosis of schizophrenia more often than the Americans. Schizophrenia was the great foible of American psychiatry. In the 1940s and 1950s American psychiatrists diagnosed it many times more frequently than their colleagues in Britain. In one study, 46 American psychiatrists and 205 British watched a videotape of "patient F," a young man from Brooklyn who had a hysterical paralysis of one arm and a history of mood fluctuations associated with alcohol abuse. Afterward, 69 percent of the Americans diagnosed "schizophrenia," 2 percent of the British.[36]

Such international differences were unscientific and embarrassing, for they suggested there was no science in psychiatry but simply the weight of national tradition, making the discipline a branch of folklore rather than of medicine.

Even worse, in the heyday of psychoanalysis many psychiatrists were simply indifferent to diagnosis, and believed that ascertaining the presumed psychodynamic cause was more important than classifying the presenting symptoms. Robert Spitzer, the Columbia psychiatrist who was the driving force behind the new system of classification introduced in American psychiatry in 1980, remembered attending annual meetings of the American Psychiatric Association in the 1960s: "The academic psychiatrists interested in presenting their work on descriptive diagnosis would be scheduled for the final day in the late afternoon. No one would attend. Psychiatrists simply were not interested in the issue of diagnosis."[37]

In the 1960s, the discipline began to wake up to the importance of getting the diagnosis right. A new generation determined that psychiatry should proceed as the rest of medicine did, establishing a differential diagnosis on the basis of the presenting symptoms, then conducting a proper investigation to come up with a clinical diagnosis, or "impression," at the end. The first wake-up call came from the Old World, or rather from the historic Viennese psychiatric tradition in exile. Erwin Stengel, born in Vienna in 1902, had studied medicine there. He knew Freud and Wagner-Jauregg, as well as being a classmate of Sakel. Yet even though Stengel had spent a year in analysis, he was not an adept of depth psychology but rather took a serious scholarly view of psychiatry as a whole. Forced to flee Vienna in 1938, Stengel landed first at the Maudsley Hospital, then in 1957 (driven, they said, from London by

Aubrey Lewis) he founded the psychiatry department of the University of Sheffield.[38]

In 1959, Stengel published an influential critique of the lack of reliability in psychiatric diagnosis. He said that both the analysts and the American followers of Adolf Meyer "stress the uniqueness of the individual." "Such an approach," he noted drily, "has tended to discourage the categorization of mental disorders."[39] This was the opening cannon in the campaign to revive diagnosis.

Ten years later, in 1969, a large comparative study of diagnosis in the United States and the United Kingdom made it clear that the two countries were badly out of sync. Commenting on the results, Heinz Lehmann said that it was time for "a renaissance of psychiatric diagnosis, which in many quarters today has deteriorated ... into an ill-regulated, superficial, unconvincing, and therefore often useless procedure."[40] Thus came from within the discipline an intellectual push for carving nature at the joints.

A second force pulling psychiatry toward a system of reliable diagnosis was the introduction of drug therapy. The availability of specific medications for psychosis and depression made diagnosis a practical matter. "With the availability of lithium and neuroleptic drugs," said Donald Goodwin and Samuel Guze, authors of an influential textbook, "distinguishing between mania and schizophrenia—once an interesting academic exercise—might now determine how a patient was treated."[41]

These issues started to become acute in the 1970s. Previously, the tradition of diagnostic classification in American psychiatry—even outside psychoanalysis—had not been glorious. The alienists of the nineteenth century worked with a few obvious categories for the sake of their annual asylum statistics (melancholia, general paralysis of the insane, dementia, and the like). American psychiatry was pushed into greater reflectiveness only by the exigencies of the census. In 1908, the Bureau of the Census asked the American Medico-Psychological Association to organize a committee on disease nomenclature, or nosology, and by 1918 the association, in collaboration with the National Committee for Mental Hygiene, had finally put together the first American psychiatric nosology, the *Statistical Manual for the Use of Institutions for the Insane*.[42]

Until this point, American medicine in general had lacked a system of nomenclature of its own. In 1927, the New York Academy of Medicine took the initiative of organizing a nationally accepted "standard nomenclature of

disease." The following year, it brought all the medical specialties together for a conference on the subject, out of which evolved the *Standard Classified Nomenclature of Disease*, published in 1933. The American Psychiatric Association had contributed the psychiatric part.[43]

At the outset of World War II, American psychiatry relied mainly on a naming system devised for patients in public asylums with major mental illnesses. Yet combat created psychiatric challenges quite different from insanity. Minor personality problems unremarkable in civilian life became of great importance in the military setting. But only such terms as "psychopathic personality" were available for understanding them.[44] Psychosomatic disorders were grouped in no single distinctive category, appearing under whatever term a gastroenterologist or cardiologist had devised (and there were many, such as "soldier's heart"). Psychological reactions to stress, combat fatigue: All demanded a new system of classification. And so new systems were devised. Psychiatric nosologies multiplied. By the late 1940s, there was chaos.

In 1948, the American Psychiatric Association's committee on naming went to work on a single national system of classification. A proposed draft was circulated to the membership, their suggestions were incorporated, and by 1952 the APA's first independent system of naming was published, the *Diagnostic and Statistical Manual [of] Mental Disorders* , known subsequently as *DSM-I*.

These details are important because a naming system incorporates the dominant philosophy of the day, and in 1952 analysts were heavily represented in the APA, on its naming committee as well as among the membership. Of the 28 current and past members in 1951 of the APA's Committee on Nomenclature and Statistics, at least 10 were members of psychoanalytic organizations or sympathetic to analysis on the basis of their writings.[45] *DSM-I* therefore codified much of their wisdom. "Psychoneurotic disorders," for example, received a straight Freudian analysis: "The chief characteristic of these disorders is anxiety which may be directly felt and expressed or which may be unconsciously and automatically controlled by the utilization of various psychological defense mechanisms (depression, conversion, displacement, etc.)."[46] More in a bow to Adolf Meyer than Freud, the term "reaction" was sprinkled liberally throughout, as in "schizophrenic reaction" or "antisocial reaction."

In the 1950s and early 1960s, psychoanalysis consolidated its hold over

American psychiatry, and the second edition of the *Diagnostic and Statistical Manual* that appeared in 1968, *DSM-II*, reflected this sway.[47] Six of the ten members of the drafting committee were analysts or belonged to sympathetic organizations. The nomenclature mirrored this predominance: Psychoneurotic problems were no longer called “reactions” but “neuroses.” The sturdy Freudian term “hysteria” appeared, replacing “conversion reaction” and “dissociative reaction.” What was hysteria? The term referred to symptoms associated with “emotionally charged situations” that are “symbolic of the underlying conflicts.”[48] Descriptions of all disorders were brief and usually lacking in operational criteria for their application. Psychiatrists seeking guidance in differentiating, for example, schizophrenia from mania would find little to help them.

Robert Spitzer, the Columbia psychiatrist who was the main architect of “DSM-III,” the diagnostic manual of the American psychiatric profession, introduced in 1980 (courtesy of Dr. Robert Spitzer).

One of the members of the drafting committee was a sort of renegade analyst named Robert Spitzer. Born in 1932, Spitzer had graduated from New York University medical school, then trained at the New York State Psychiatric Institute, simultaneously qualifying as an analyst at Columbia's Psychoanalytic Clinic. Around 1959, however, he abandoned the analytic pathway to join the classifiers, and by 1968 he had become the head of the Evaluation Unit of the New York State Psychiatric Institute.

In these years, Spitzer decided to take psychiatric diagnosis in another direction entirely, making the diagnoses as precise as possible in order to correspond to what were presumably natural disease entities. This was known as "cutting nature at the joints," and was precisely what Kraepelin had set out to do many years previously. Yet where the joints lay exactly could only be determined by research. The best way to do research would be to devise operational criteria for determining exactly what symptoms must be present before a physician could make a given diagnosis.

Fortunately for Spitzer, elsewhere on the American psychiatric landscape other small groups of researchers were setting out to carve nature at the joints as well. In 1948, Eli Robins, a Texan with a Harvard MD who had trained at the Massachusetts General Hospital, accepted a fellowship in psychiatry in Washington University of St. Louis. As he arrived, he encountered two young residents, George Winokur and Samuel Guze, whose interests were very similar to his own: brain chemistry, biology, and classification—all at the time highly unfashionable subjects. The three of them became a nucleus from which a counterrevolution in psychiatry against psychoanalysis would ultimately emanate. Collectively, they became known as the "Neo-Kraepelinians,"[49] a not entirely accurate label given Kraepelin's uninterest in brain biology. In 1955, Guze became chair of the department and proceeded to give it an international reputation as a center of biological thinking, fostering such researchers as Paula Clayton (coauthor with Winokur of a textbook on psychiatry's medical basis[50]), John Feighner ("Finer," who became an important diagnostician), and Robert Woodruff (a student of the biology of "hysteria").

In 1972, the St. Louis group, led by Feighner, published the first set of rigorous criteria for making diagnoses, in which one would be guided not just by one's "best clinical judgment and experience" but by fixed conditions the patient had to meet. Was the patient depressed? He or she would have to have (a) a dysphoric mood; (b) at least five of a list of eight criteria, including poor

appetite, feelings of guilt, and so forth; (c) a psychiatric illness that had gone on for at least a month before seeking help. Similar sets of criteria were spelled out for the entire gamut of psychiatric disorders.[51] The "Feighner criteria," or "Research Diagnostic Criteria" as Spitzer refined them,[52] were revolutionary in permitting the standardization of diagnosis from clinician to clinician, university to university, country to country.

Meanwhile, the American Psychiatric Association was under pressure to revise *DSM-II.* Gay groups were unhappy that homosexuality had been included as a disorder.[53] Insurance companies wanted more precise diagnoses if they were going to pay for long-term psychotherapy. Many clinicians were becoming disaffected from psychoanalysis and wanted diagnoses based on symptoms rather than on dubious theories about underlying causes. Early in 1973, Walter Barton, medical director of the APA, initiated a task force "to revise *DSM-II* and prepare *DSM-III* within the next two years."[54] Melvin Sabshin, a Young Turk, then succeeded Barton as medical director. Sabshin realized that if the group who had designed *DSM-II* were to take on the revision, *DSM-III* would become just "a minor variant" of its predecessor. What was needed was something completely different. In April 1974, Sabshin summoned Spitzer and Theodore Millon, another Young Turk who was a PhD psychologist at the Neuropsychiatric Institute of the University of Illinois Medical Center in Chicago, for an all-day conference.[55] Out of this conference came the leadership team that would drive forward *DSM-III*, published in 1980.

Spitzer headed the task force. Also serving on it were Clayton and Woodruff from the Guze group (a third of the task force had trained at Washington University); Donald Goodwin at the University of Kansas who had studied with Guze was on it, as was Z. J. ("Bish") Lipowski, a specialist in delirium, which is an organic psychiatric condition. There was Donald Klein, a psychopharmacologist and psychiatrist who in 1978 became professor of psychiatry at Columbia, as well as 13 other members. Just as previous DSM task forces had been weighted in favor of psychoanalysis, this one was weighted against it and toward biological psychiatry, though the members did not use that then inflammatory term. As Spitzer later said, "With its intellectual roots in St. Louis instead of Vienna, and with its intellectual inspiration derived from Kraepelin, not Freud, the task force was viewed from the outset as unsympathetic to the interests of those whose theory and practice derived from the psychoanalytic tradition."[56]

There is no doubt that the task force intended to produce a science-driven document. As Spitzer said in the introduction to *DSM-III*, "In attempting to resolve various diagnostic issues, the Task Force relied, as much as possible, on research evidence relevant to various kinds of diagnostic validity." Between 1977 and 1979 the National Institute of Mental Health sponsored a field trial in which 500 psychiatrists from many different centers used drafts of *DSM-III* in diagnosing over 12,000 patients. Around 300 psychiatrists were paired, their evaluations then compared for consistency.[57] Whereas *DSM-II* contained 134 pages, *DSM-III* ran on to almost 500 pages, each page containing long lists of criteria that had to be met before a given diagnosis could be made.

So the spirit of science was there. It was on the basis of Kety's Copenhagen adoption study, for example, that Spitzer proposed the diagnosis "schizotypal personality disorder," a certain type of personality occurring in individuals with a family history of full-blown schizophrenia.[58] Harvard psychiatrist Gerald Klerman called *DSM-III* "a victory" for science. [59] Indeed, on the basis of the presumed scientific underpinning of *DSM-III*, American psychiatry returned to the world of medicine, applying the medical model in diagnosis and downplaying the vague "biopsychosocial model" under which so much mischief had occurred. In the judgment of one historian of psychiatry, *DSM-III* became "the centerpiece of the knowledge base of the profession."[60]

This repositioning of American psychiatry had a great impact on events elsewhere. By the early 1990s, *DSM-III*, or the revised version of it that appeared in 1987 *(DSM-III-R)*, had been translated into over 20 languages. French psychiatry residents, initially taken with antipsychiatry and the doctrines of Jacques Lacan and Michel Foucault, began memorizing the 4 criteria (and the 18 possible symptoms, 6 of which must be present) for anxiety disorder.[61] In Germany, the translation of *DSM-III-R* reawakened Kraepelinian memories of psychiatric "diseases" *(Krankheiten)* and not just "disorders."[62] The appearance of DSM-III was thus an event of capital importance not just for American but for world psychiatry, a turning of the page on psychodynamics, a redirection of the discipline toward a scientific course, a reembrace of the positivistic principles of the nineteenth century, a denial of the antipsychiatric doctrine of the myth of psychiatric illness.

Yet there were nagging worries. Rather than heading off into the brave new world of science, DSM-style psychiatry seemed in some ways to be

heading out into the desert. For one thing, with each successive edition of the DSM series, the number of discrete disorders kept growing. *DSM-III* listed 265 different disorders, up one third from the 180 in *DSM-II*. *DSM-III-R* had 292 and *DSM-IV*, published in 1994, 297 disorders.[63] Did nature really have 297 joints? Even though the drafters shunned the Kraepelinian label "disease," at bottom identifying distinct "disorders" meant distinct disease entities.[64] One would not expect to find so many different diseases in nephrology or cardiology. Of course the brain is more complex, but even so, Pinel had managed to reduce the number of psychiatric disorders from "an indefinite number of varieties" to four.[65] Psychiatry had a tradition of intellectual compactness, of lumping rather than splitting. One of the *DSM-III* drafters justified wide inclusiveness on the grounds of wanting to "embrace as many conditions as are commonly seen by practicing clinicians," in order to let future researchers judge the validity of those conditions as "a valid syndromal entity."[66] Yet the sheer endlessness of the syndrome parade caused an uneasy feeling that the process might somehow be out of control.

DSM-III also seemed highly ethnocentric, a major flaw in a discipline that presumes to universality. Many of its disorders, such as anorexia nervosa, were simply unknown in other parts of the world. One can be sure that, if *DSM-III* had been drafted in India, it would have included a big section on demonic possession.[67] Was it possible that the brave new world of psychiatry was bogging down in "borderline personality disorder" and the like—read "Woody Allen syndromes"—generated by very specific East-Coast cultural pathology? As one critic said, "Borderline and narcissistic personalities are rarely seen in Iowa City or in Mobile; certainly, they are not recognized in Tangiers or Bucharest."[68] For two centuries, culture had proven to be a swamp for psychiatry; witness the "ovarian hysteria" of the nineteenth century, the "colonic autointoxication" —born of an urban middle class's bowel obsessions—of the early twentieth. If supposedly scientific psychiatry was merely trying to classify the cultural preoccupations of the North American middle class, its naming manuals would soon be as outdated as the etiquette guides for Victorian ladies.

Politics represented a final pothole on the high road of science for the *DSM-III* drafters. Even though they were struggling to cling to "the data," they were buffeted by ideological lobbies and forced to make a series of concessions. All this negotiating left the impression that what the drafters had created was as much a political document as a scientific one. First, there was

the squabble about removing homosexuality from the illness list. *(DSM-II* had called homosexuality a "sexual deviation." [69]) A subcommittee of the *DSM-III* task force deliberated about calling it "homodysphilia, dyshomophilia, homosexual conflict disorder, amorous relationship disorder ... and finally, ego-dystonic homosexuality." Unable to agree, the subcommittee kicked it upstairs to the task force members, who decided to delete it entirely.[70] A referendum of the APA membership confirmed the decision in 1974. At a stroke, what had been considered for a century or more a grave psychiatric disorder ceased to exist.

The political power of the psychoanalysts also shaped the structure of *DSM-III*. The analysts were outraged when they saw in the task force's first draft, completed in 1975, that disorders were to be based on cause "when known," avoiding "unproven etiologic concepts." The analysts argued that decades of experience in analytic practice constituted proof of Freudian-style psychogenesis. Nor were they pleased that "neurosis" had vanished from the task force's draft. Faced with the analysts' threat to derail the entire enterprise at the meeting of the APA's Board in May 1979, the task force pulled back and restored "neurosis" in parentheses after the word disorder.[71] The drafters somewhat lamely explained in the introduction that they meant neurosis "only descriptively," without implying a "neurotic process."[72] This clearly political sop to a major interest group had nothing to do with science.

In the years after 1971, the Vietnam veterans represented a powerful interest group. They believed that their difficulties in reentering American society were psychiatric in nature and could only be explained as a result of the trauma of the war. In language that anticipated the "struggle for recognition" of numerous later illness attributions, such as repressed memory syndrome, the veterans and their psychiatrists argued that "delayed massive trauma" could produce subsequent "guilt, rage, the feeling of being scapegoated, psychic numbing, and alienation." In early 1973, the National Council of Churches organized a First National Conference on the Emotional Needs of Vietnam-Era Veterans. Out of this grew a nationwide campaign to persuade the recalcitrant psychiatric establishment to recognize the new disease. Once it became known how easily the APA's Nomenclature Committee had given way on homosexuality, it was clear that the psychiatrists could be rolled. A "working group" of psychiatric advocates of the veterans' cause suggested the diagnosis "post-combat disorder," and as the working group began meeting with Spitzer and a task force subcommittee

on “reactive disorders” that he had appointed, it became inevitable that there would be agreement: In 1978, the committee on reactive disorders recommended to the task force the inclusion of a diagnosis of “post traumatic stress disorder,” or PTSD, in the new manual. One student of this campaign concluded, “PTSD is in *DSM-III* because a core of psychiatrists and veterans worked consciously and deliberately for years to put it there. They ultimately succeeded because they were better organized, more politically active, and enjoyed more lucky breaks than their opposition.”[73]

The experiences of the homosexuals and the Vietnam veterans made it evident that psychiatric diagnosis was up for grabs. The next group to pressure the naming committee was the feminists, unhappy that *DSM-III-R*, published in 1987, contained listings such as “self-defeating personality disorder,” said to be commoner in females than males by a ratio of two-to-one. The committee put this in a category of new disorders “needing further study.” Nor were feminists enchanted by “late luteal phase dysphoric disorder,” also in the category of “needing further study.”[74] It was not as a result of further study but of political pressure that self-defeating personality was dropped from *DSM-IV*, published in 1994. Menstrual problems continued on the list for study as “premenstrual dysphoric disorder.”[75]

Given such antics, it would be difficult to take seriously any official psychiatric pronouncement about problems surrounding sexual orientation, the psychiatry of stress, or women and the menses. These matters could all apparently be pathologized and depathologized at the will of the majority, or following campaigns of insistent pressure groups. The underlying failure to let science point the way emphasized the extent to which *DSM-III* and its successors, designed to lead psychiatry from the swamp of psychoanalysis, was in fact guiding it into the wilderness.

The Decline of Psychoanalysis

In the background of the evolution of the DSM series lay the ongoing decline of psychoanalysis. DSM-IV dropped the term neurosis because the new task force realized that it now had enough votes to get away with it. As psychiatric historian Mitchell Wilson said of the psychoanalysts in the 1970s, “The balance of power within American psychiatry had been shifting under

their very feet. What had been the new, modern psychiatry only two decades earlier had become ... an encumbrance too unwieldy for [1970s psychiatry] to bear."[76]

Yet ironically, the fall from grace of psychoanalysis contributed to further disorientation and confusion with psychiatry rather than freeing it up for a walk toward science in the way that the decline of humoral theories in the early nineteenth century had freed up pathology for such a grand promenade. With the transformation of psychoanalysis into yet another postmodern form of "discourse," competing psychotherapies mushroomed, so that by the 1970s one could count at least 130 different versions.[77] In the psychotherapeutic jungle that arose, any therapy was considered valid no matter how bizarre, from "therapeutic touching" to practicing being born again midst the sofa cushions on the living-room rug. For psychiatrists, touching these was like touching a coiling mass of serpents.

The decline of psychoanalysis had a disorienting effect in particular on the "biopsychosocial" wing of psychiatry, which hitherto had depended on Freud's ideas for most of its psychological understanding. If not Freud, then what do we offer for psychotherapy? The alternative psychotherapies all had similar success rates. It really didn't seem to matter whether one chose interpersonal therapy, group therapy, family therapy, psychodrama, hypnosis, or narcosynthesis. The patient's chances of recovery were mostly the same.[78] Even more unnerving, as psychologist Hans Eysenck pointed out, was that statistical studies suggested none of these therapies improved significantly on chance.[79] Is it any wonder therefore that in the 1970s and beyond, the psychotherapeutic side of psychiatry began wandering in the wilderness, a paradoxical result of the decline of psychoanalysis, the one psychotherapy that had formerly given therapists great certainty.

Old hands were shocked at the changes. When Robert Wallerstein was a resident at the Veterans Administration Hospital in Topeka, Kansas (then under the sway of the Menninger Clinic), psychoanalysis was "*the* road to the treatment of mental disorder." His residency training consisted mainly of sessions in depth psychotherapy. In a resident's average workweek of 40 hours, 20 would be given to "individual psychotherapeutic work with patients." Twenty hours a week times 50 weeks times three years equaled "3,000 hours of psychotherapy during the residency training period."

Since 1949, noted Wallerstein, there had been vast changes in the training of young psychiatrists. In a typical four-year residency today, the major

rotations are “no longer psychotherapy focused but rather are drug-management focused.” Because the patients rarely stay longer than a month, the residents have no opportunity to try out long-term psychotherapy. Today’s psychiatric resident has to attend clinics at community mental health centers, eating disorder clinics, sexual disorder clinics, drug maintenance clinics, “all with their drug treatment and behavior treatment focus.” All of this training is taken from the time once allotted depth psychotherapy, so that today’s resident would typically log about 200 hours of practice in psychotherapy. “Given that the total 4-year residency program constitutes up to 8,000 scheduled hours, and that the 200 psychotherapy experience hours constitute only 2 and one-half percent of that time, one can well ask....”[80] Although Wallerstein left the question dangling, the implications were clear. In the passage of psychoanalytically oriented psychotherapy from 50 percent to 2.5 percent of the graduate curriculum may be traced the decline and virtual demise of psychoanalysis within psychiatry.

The analysts lost control of the great bastions of psychiatric training. In 1945, said Bertram Brown, director of the National Institute of Mental Health, “it was nearly impossible for a nonpsychoanalyst to become chairman of a department or professor of psychiatry.” By the mid-1970s dedicated analysts were rarely being selected as department chairs.[81] By 1990, more than 100 of the 163 residency training programs for psychiatry in the United States had abandoned instruction in intensive psychotherapy. “More than 60 percent of the psychiatrists now in training,” said one observer, “may complete their training without ever seeing a patient in twice-a-week or more intensive psychotherapy.”[82]

With the decline of analysis, the whole social experience of becoming a psychiatrist changed, from preparation for a white middle-class society practice to preparation for the real world. The programs of the 1940s and 1950s were dedicated to training “cream puff” psychiatrists, as Paul Hoch, the tough-minded senior research scientist at the New York State Psychiatric Institute, put it: “men and women oblivious to the harsh realities facing millions of Americans ... content to live out their professional lives dedicated to the welfare of a thin layer of well-heeled patients at the top of the economic heap.”[83] When in 1955 the Institute’s new director Lawrence Kolb decided to stop admitting only “interesting” patients, meaning those suitable for psychoanalysis, and to open the doors of the hospital to the local population of Manhattan’s Washington Heights district, a dramatic change

occurred in the patients, who went from being white and Jewish to black and Protestant.[84]

As the academic training programs lost their following, the supply of psychiatrists interested in becoming psychoanalysts dwindled. The percentage of psychiatrists who were also analysts declined from around 10 percent in the 1950s to 2.7 percent by 1988.[85]

Simultaneously, psychoanalysis was losing its grip in the world of psychiatric practice. When Fritz Redlich and coworkers surveyed psychiatric practice in New Haven in 1950, they found psychoanalysis everywhere in ascendancy, used on 32 percent of all patients and an object of "universal acclaim" among the residents at Yale. When Redlich went back 25 years later, psychiatrists who did not prescribe drugs "had become the exception," and group and family therapies had outdistanced depth psychotherapy. "The major emphasis was on symptom relief ... rather than on achievement of personality insight or change," said Redlich.[86] At the Menninger Clinic in Kansas—once a bastion of psychoanalysis—by 1965 only one patient in 209 had been referred for psychoanalysis and only 23 percent were participating in any kind of psychotherapy (down from 62 percent receiving psychoanalysis or psychotherapy in 1945).[87]

Former chieftains such as Ludwig Binswanger, Freud's Swiss disciple, began falling away: A relative of Binswanger's with a psychiatric illness was successfully treated by a young, pharmacologically oriented German psychiatrist named Fritz Flügel. At the end of the treatment, Binswanger told Flugel, "Fritz, with two pills, you destroyed a psychodynamic castle that took me 50 years to build."[88] The Binswanger clinic at Kreuzlingen, once a fortress of Binswanger's own depth "existential analysis," closed its doors in 1980.

Sniffing calamity, in 1974 the American Psychoanalytic Association polled its membership to see what was happening on the ground. Alternative psychotherapies, such as group and family therapy and the "human potential movement" were tearing away at the patient base. "Self-help measures such as biofeedback, meditation, autohypnosis, imagery, and alternative styles of life are held to promise great benefits to their users. In short, two decades ago when one was dissatisfied with his life, he had a limited choice as to what to do about it. Now he can turn in many directions, only one of which is psychoanalytic therapy." Three-fifths of the Association's members had started prescribing medications, a further third were offering marital therapy

or some variant. “There are fewer good classical neurotics,” the report lamented. Along with the schizophrenogenic mother, the housebound, neurotic, frustrated female who had been the darling of the couch was also vanishing, replaced by “women patients with achievement conflicts.”[89]

Finally, even the patients themselves lost faith. The American Psychoanalytic Association’s 1974 poll of its membership showed significant declines in the number of patients seeking full-blown analytic treatment compared with the period 1962-1967: Earlier, almost half of the association’s members were treating four to seven analytic patients per week; by 1976 this was down considerably. Previously, a third of the members were treating eight or more patients per week. Now, complained the authors, “only one-fifth of our members had such thriving analytic practices last year.”[90] The reluctance of Health Maintenance Organizations, or HMOs, the new wave in American health care, to pay for long-term therapy helped account for part of the decline, but only part: The demand for analysis was receding.

In cinema, images of psychoanalysis went from the idolatrous, such as John Huston’s *Freud* (1962) or *Captain Newman MD* (1963, a film about Los Angeles analyst Ralph Greenson, who later treated Marilyn Monroe) to films depicting analysts as sex-mad letches (*The Girl in the Freudian Slip*) or domestic stumblebums (*The ImpossibLe Years*, a 1968 film starring David Niven and Ozzie Nelson). It was joked that in the movies “bad psychiatrists have sex with their patients and then murder them while good psychiatrists simply have sex with them.”[91] A 1993 *Time* cover story on psychoanalysis showed Freud with his cranium crumbling into pieces of a picture puzzle; an inside photograph had an analytic couch flying out the window. “What if Freud was wrong?” the magazine asked a presumably unsettled readership.[92] Unlike the sleek Berliners of the 1920s discussing their “Minko’s,” the American middle classes of the 1990s had moved onto other themes. The “P” word was no longer psychoanalysis. It was Prozac.

A 1980s court case represented a final humiliation for analysis as a medical therapy. The widely followed case pitted a depressed patient, Rafael Osheroff, against the psychoanalytically oriented Chestnut Lodge private hospital. In 1979, Osheroff, a 42-year-old physician from Alexandria, Virginia, was admitted to Chestnut Lodge with the symptoms of psychotic depression. In the course of his seven-month stay at “the Lodge” he was treated with four sessions in intensive psychotherapy a week and denied medication despite his own requests, on the apparent grounds that his

clinicians wanted him to regress back to the point in childhood at which the initial trauma occurred and then “build” from there.[93] Dr. Osheroff, by contrast, merely wanted to get better and finally obtained a transfer to another private clinic, the Silver Hill Foundation in Connecticut, where he was treated with phenothiazines and antidepressants. Within three months, he was discharged and able to return to normal life. On returning home, Osheroff found that his world had disintegrated, his wife having left him, his hospital accreditation gone, and his partner (the man who had driven him to Chestnut Lodge) having ousted him from their joint practice.

In 1982 Osheroff sued Chestnut Lodge for malpractice on the grounds that he should have been given state-of-the-art treatment with medications of demonstrated efficacy rather than left to vegetate for seven months. An arbitration panel awarded him $250,000. Both the Lodge and Osheroff appealed, and in 1987 the case was settled out of court for an undisclosed sum just before coming to trial. Did psychoanalysis meet “accepted standards of care”? Gerald Klerman, an influential psychiatrist then at Harvard who had testified on Osheroff’s behalf, made the point that the efficacy of intensive psychotherapy had never been established with controlled trials, whereas that of medication had, and that controlled trials were the ultimate standard of science.[94] The case left the strong impression that treating major psychiatric illnesses with psychoanalysis alone constituted malpractice.

The Osheroff case set the leaves to rustling. Even though a court had not definitively ruled that biological and behavioral treatments constituted the new gold standard of care, any clinician who henceforth treated patients as Chestnut Lodge had Dr. Osheroff ran the risk of incurring heavy penalties.

Thus psychoanalysis began to demedicalize itself. As medicine lost confidence in it, nonphysicians began filling the vacancies in the analytic training programs. For many years, American psychoanalysts had strenuously resisted accepting non-MDs in psychoanalytic training. In 1985, four psychologists filed a class-action suit against the American Psychoanalytic Association, arguing that its training monopoly violated federal antitrust law. In 1988, the various institutes agreed to begin admitting nonmedical candidates. At this point, social workers and psychologists began to pour into psychoanalytic training programs: By 1991, 21 percent of the training candidates were nonmedical.[95]

And why not? Increasingly, the view became accepted that psychoanalysis was not for illness but for the interior voyage. While insisting that

psychoanalysis was still valid therapy for “the major psychoses,” analyst Robert Michels decided to take a more embracing stance: The discipline was ideal for “the optimalization of experience and the enhancement of sensitivity.”[96] Indeed, said critic Adolf Grunbaum, picking up on comments that Michels had made elsewhere, analysis was most akin to “an edifying experience of the kind provided by, say, a season ticket to the opera.”[97]

From *Studies in Hysteria* in 1895 to a ticket to the opera in 1994: what an odyssey! By the mid-1990s, psychoanalysis had by no means gone out of fashion among intellectuals, and it was a rage in many departments of languages and sociology. In 1994, the University of Dublin began offering an undergraduate arts degree in psychoanalysis. [98] The late-twentieth-century trajectory of psychoanalysis had carried it beyond the discipline of psychiatry and into the ether of arts and letters where, however it fared, it would no longer be identified as a privileged treatment for psychiatric illness.

What had gone wrong for psychoanalysis? There were external factors, such as the changing demographics of part of its social base (see pp. 307-308). But internal ones mattered as well. The demise of psychoanalysis was in large measure a result of its own lack of flexibility, its resistance to incorporating new findings from the neurosciences. And this reluctance was directly related to the analysts’ fear of being proven wrong.

American psychoanalysis had always exhibited strenuous resistance to the collection of data on the outcome of therapy. In 1948, the American Psychoanalytic Association set up a committee to evaluate psychoanalytic therapy. The committee met with “insuperable resistance among the membership,” and the idea was permitted to lapse.[99] At the Association’s May meeting in 1952, President Robert Knight decided to try again: “The highly individualistic nature of psychoanalytic practice ... [has] made the gathering of statistical data from psychoanalytic experience exceedingly difficult in the past. Analysts have tended to hide behind the curtain of professional secrecy and have been loath to participate in sufficient numbers ... ,” particularly in research on the “outcome of therapy of patients under psychoanalytic treatment.” But Knight was determined to turn this around, and in 1953 he and Lawrence Kubie prompted the Association to organize a Central Fact Gathering Agency.[100] The committee worked in secretiveness, and was instructed that before any information could be presented “to any group outside of the Association” the executive council must first approve. [101] By the time that Kubie reported on the committee’s progress in April

1956, the grumblings had grown loud indeed: The committee's statistical categories were, it appeared, much too coarse "to express the shadings, degrees, and grades of thinking relative to mental illness." Kubie urged the members to be patient: "Nothing which the American Psychoanalytic Association had ever undertaken as an association had aroused as much enthusiasm among outsiders interested in medical and psychiatric education as this research project of the Central Fact-Gathering Committee. The project may be expected to elicit financial backing from foundations...."[102] It is clear, therefore, that the ultimate rejection of psychoanalysis by researchers interested in scientific standards did not happen without a long period of suspending disbelief, indeed of benevolent encouragement.

And yet the committee brought it to nothing. In December 1957, chair Harry Weinstock told the annual meeting that the committee had encountered great difficulties. Only a few results would be circulated. "Because of ... the potentially misleading nature of the statistics it is recommended that members keep this report confidential." The committee was disbanded.[103] What then happened was even more piteous: The 3,000 reports from members plus follow-up data on outcomes of their analyses were sent to IBM for keypunching. The original data were lost. Some of the codes permitting the interpretation of the punchcards were lost. Some of the punchcards themselves were lost. Much later, in 1967, the Association managed to stagger together a few anodyne factoids and the whole project of "evaluating" the results of psychoanalysis was abandoned. [104] The entire fiasco was a monument to the psychoanalysts' fears of statistical verification.

With good cause. The few statistics that outsiders had managed to assemble spoke quite negatively of the effectiveness of psychoanalysis compared with other psychotherapies. In 1952, Hans Eysenck, director of the psychological department of the Maudsley Hospital, became the first to compare the outcomes of psychoanalysis with other "eclectic" therapies. Pointing out that two-thirds of all neurosis patients recover spontaneously within two years, Eysenck found that only 44 percent of psychoanalytic patients had improved by the end of their analysis, compared with 64 percent of those treated with eclectic therapies.[105] Even the eclectic therapies had not improved on chance, whereas psychoanalysis had, if anything, impeded the recovery of its clients.

In the coming decades, expressions of scientific disbelief in psychoanalysis mounted. A 1954 textbook by William Mayer-Gross, a Heidelberg

psychiatrist (and Nissl student) who had found refuge in England, charged that “Freud’s superficially rational appeal, made under the cloak of science, is probably the most effective form of faith-healing today.”[106] Although Mayer-Gross had scorned the Americans for their curious obeisance to Freudianism, it was above all in the United States that the hammer blows against analysis began to ring out.

An example: In 1959 Donald Klein was still at the Hillside Hospital. He had a group of very anxious, panicky patients whom he put on the new drug chlorpromazine. Nothing happened. Wait a minute, Klein reasoned. American psychoanalytic theory said that psychic problems were caused by anxiety, itself the result of intrapsychic conflicts. Schizophrenics, who often were very anxious, did get better on chlorpromazine. But anxiety patients did not. Hence, it was probably not true that simple anxiety was just a few degrees down the anxiety scale from schizophrenia. “The continuity theory, which held that psychosis was simply a quantitative spilling over of the same anxiety suffered by people with more moderate disorders, could not be correct. If anything, the situation implied a physiological discontinuity, as with pneumonia and colds.” Penicillin worked for pneumonia but not for colds. “This fact alone should lead one to guess that pneumonia and colds are not simply quantitative variants of the same theme, but are qualitatively distinct illnesses.”[107] There must, in other words, be separate disease entities in psychiatry, not just a sliding scale from wellness to neurosis, to psychosis.

How about the psychoanalytic approach to schizophrenia? In the early 1960s, two researchers at the Camarillo State Hospital in California compared the length of stay of various groups of first-episode schizophrenics. One group was treated with psychoanalytic-style psychotherapy, another with drugs, still others with various combinations of therapy, or—the control group—with nothing. Those who received psychotherapy alone did least well (an average stay of 191 days). In fact, they did worse than the control group who received no treatment. Those who received the antipsychotic drug Stelazine (trifluoperazine) with no psychotherapy were out in 151 days.[108] For followers of Harry Stack Sullivan and Frieda Fromm-Reichmann, these were not encouraging results.

In the 1970s and after, the retreat of psychoanalysis turned into a rout. Concluded psychologists Seymour Fisher and Roger Greenberg in 1977, “There is virtually no evidence that therapies labeled ‘psychoanalysis’ result in longer-lasting or more profound positive changes than approaches that are

given other labels and that are much less time-consuming and costly."[109] In two large tomes published in 1984 and 1993, philosopher Adolf Grunbaum of the University of Pittsburgh exposed the nullity of such notions as "transference," the core therapeutic concept in analysis. [110] Scholar after scholar scored the analysts for substituting essays and anecdotes for clinical data capable of scientific validation. One 1995 opus with 75 pages of footnotes gently explained *Why Freud Was Wrong*.[111] The Oedipal complex, infantile sexuality, the nature of dreams, women's special sexual attributes: All simply became objects of disbelief, not so much disproven—for they were incapable of disproof—but relegated to the same scientific status as astrology. Mainline psychiatry and psychology lost interest in psychoanalysis, stopped citing contributions in psychoanalytic journals, and pressed on in the study of thinking, memory, perception, and dysfunction as processes driven by a genetic program rather than by "the pressure of instinctual drive gratification invoked by Freud," as Grünbaum put it.[112]

"All sciences have to pass through an ordeal by quackery," observed Eysenck in 1985. "Chemistry had to slough off the fetters of alchemy. The brain sciences had to disengage themselves from the tenets of phrenology ... Psychology and psychiatry, too will have to abandon the pseudo-science of psychoanalysis ... and undertake the arduous task of transforming their discipline into a genuine science."[113]

Cosmetic Psychopharmacology

When Peter Kramer, a psychiatrist at Brown University, coined the delicious phrase "cosmetic psychopharmacology" in 1990,[114] he was applying it to a new antidepressive drug the Eli Lilly Company had developed called Prozac (fluoxetine). Kramer came in for much scorn in hyping a drug that, he claimed, made patients feel "better than well."[115]

Yet the symptoms of depression and anxiety that Prozac and similar drugs treated were not trivial affairs. The NCS study found that within the previous 12 months, 10.3 percent of all Americans had a major depressive episode, and that on a lifetime basis 19 percent of the population—1 person in 5—would undergo an often-disabling mood disorder. Almost 20 percent of the

entire population had experienced a phobic disorder such as agoraphobia, fear of open spaces, within the previous 12 months; and almost 1 in 20 would feel a generalized kind of anxiety.[116] Such conditions were, in other words, extremely common, and those suffering from them were entitled to help rather than ridicule for their failure to pull up their socks.

There is no doubt that psychotherapy helped patients feel more comfortable with their psychiatrists, in contrast to the old-style alienists with their lack of interest in the complexities of the human spirit. Yet lifting symptoms rather than cultivating a sympathetic rapport in the office remained the ultimate therapeutic objective. And problems that had their origins in the brain would find their remedy in drugs that acted on the brain's chemical receptors. This was the good news for psychiatry: that a new panoply of drugs made it possible to help people with garden-variety anxiety and dysphoria, as opposed to the major psychiatric illnesses for which the antipsychotics, tricyclic antidepressants, and antimanic drugs stood ready.

The bad news was that by the end of the twentieth century these drugs had acquired such currency that, much as in the eighteenth century, patients began to view physicians as mere conduits to fabled new products rather than as counselors capable of using the doctor-patient relationship itself therapeutically. With the introduction of cosmetic psychopharmacology, medicine had come full circle. In olden times, patients had often been impatient with physicians, seeing them mainly as boil-lancers and enema-givers whose main resource lay in being able to write a prescription for a very powerful laxative, the kind of therapy that traditional patients coveted. In the world of postmodern medicine as well, patients often resented doctors for their real or imagined shortcomings, and saw the consultation mainly as a way of getting a prescription for drugs that they, the patients, had already predetermined were the answer to their problems. Physicians in primary care experienced this drug-seeking around requests for penicillin and other antibiotics.[117] In psychiatry it was experienced as patients' demands for Valium and Prozac.

Psychoactive drugs have always been available in one form or another to help people deal with depression and anxiety. Alcohol, which acts initially as a stimulant then a depressant, is as old as time. Opium achieved currency in the eighteenth century, and its alkaloids were used medically for depression in the nineteenth. The barbiturate sedatives had been available since the turn of our own century. Yet all had disadvantages in terms of addiction, daytime

sedation, and inability to lift the core symptoms of psychiatric disorder.

The story of cosmetic psychopharmacology, the use of drugs with relatively few side effects to lift quotidian anxiety and depression, began with the advent of Miltown. Frank Berger, a Jewish refugee from Hitler, was the architect of this tale. Berger was born in Pilsen, Czechoslovakia, in 1913 and graduated in medicine from Prague in 1937. He fled to England and tossed about during the war as a bacteriologist, going to work in 1945 for the firm British Drug House. There he did some work with a muscle-relaxant called mephenesin, thinking that it might help patients with Parkinsonism. It did not, but Berger noted that it reduced anxiety for very short periods. “I was interested in the neuropharmacological basis of mental disturbances,” said Berger later. “The phenomenon that interested me most was the physiological basis of nervousness. Most people get nervous and irritable for no good reason. They flair up, do not differentiate between serious problems and inconsequential ones, and somehow manage to get excited needlessly. These people are not insane; they simply are over-excitable and irritable, and create crisis situations over things that are unimportant. What is the physiological basis of this overexcitability?”[118] That turned out to be the billion-dollar question.

In 1947, Berger emigrated to the States, becoming assistant professor of pediatrics at the University of Rochester, and consulting for a small drug house named Carter Products whose chief previous claim to fame had been “Carter’s Little Liver Pills.” Carter wanted Berger to develop a mephenesin-like product for anxiety, and the firm instructed their crackerjack organic chemist, Bernie Ludwig, to synthesize some new compounds. In May 1950, he produced one that was later given the generic name meprobamate. Yet Carter lost interest in the drug after polling a sample of physicians and asking them if they would use a drug that acted against anxiety. Most said no. Meanwhile, in 1949 Berger had joined Carter’s subsidiary Wallace Laboratories in Wallace, New Jersey. Berger had confidence in meprobamate and, with his executive authority at Wallace, went through all the steps of working meprobamate up, arranging trials for a thousand patients, giving it to pregnant animals to see if it caused birth defects, even making a movie to show how meprobamate calmed Rhesus monkeys, who usually are very angry about being in captivity.

Some people from Wyeth Laboratories saw the movie, expressed their interest to Berger, and learned that Carter Products wanted to sell the license.

In 1955, Wallace began marketing meprobamate as “Miltown,” Wyeth as “Equanil.”[119] Both names went on to have enormous resonance in American cultural history in the 1950s as “tranquilizers.”

Within psychiatry, there was intense excitement about Miltown. At the May 1955 meeting of the American Psychiatric Association, a “whispering campaign” went on, possibly instigated by Wallace Labs. “Have you heard of Miltown? I hear it’s terrific.” As veteran psychopharmacologist Frank Ayd recalled, “Few psychiatrists or science writers returned home from the 1955 APA meeting unaware of the existence of meprobamate.”

In the following months, the demand for Miltown and Equanil was far greater than for any drug ever marketed in the United States. The supply in drugstores soon ran out and pharmacists would put signs in the window reading “out of Miltown” or “Miltown available tomorrow.”[120] Miltown became a household word when television-host Milton Berle started humorously referring to himself as “Miltown” Berle. The title of S. J. Perelman’s 1957 book *The Road to Miltown* made the drug common coinage among the Book-of-the-Month set.[121] *Look*, *Christian Century*, *Today’s Health*, and *Time* ran stories about “Happy Pills,” “Happiness Pills,” “Peace of Mind Drugs,” and “Happiness by Prescription.”[122] By 1956, one American in twenty was taking tranquilizers within a given month.[123] Miltown was thus the first psychiatry drug to become the object of a popular frenzy.

Valium was the next frenzy.[124] Competing pharmaceutical houses had been observing intently the emergence of chlorpromazine and Miltown. In 1954, Hoffmann-La Roche, a Swiss-based drug house with a large American office in Nutley, New Jersey, instructed its organic chemists to develop a “psychosedative drug.”

Interestingly, neither university scientists nor government grants were involved in any of this: It was all driven by the profit motive. As Irvin Cohen, one of the psychiatrists who first tested Valium’s sister-drug Librium later reflected, “The benzodiazepine [Valium etc.] story is essentially a model of how a therapeutic agent is conceived and brought forth had further potential. He was sent back to the lab bench. In 1959, he came up with a related benzo, diazepam, that was considerably more potent and that could be stabilized in pills. Roche marketed diazepam in 1963 as “Valium,” which until the introduction of Prozac was the single most successful drug in pharmaceutical history. In 1969, Valium surpassed by an enterprising pharmaceutical manufacturer who simply seeks to find a drug superior to others already in

the marketplace.“[125] Thus Roche was merely hoping that its organic chemists would bring it abreast of the game.

Leo Sternbach, who in the late 1950s synthesized the first of the benzodiazepine tranquilizers (“Librium,” “Valium”), here seen in his lab at Roche Laboratories in Nutley, N. J. Valium was introduced in 1963, a year after the photograph was taken (courtesy Hoffmann-La Roche).

One of Roche's chemists was a refugee named Leo Sternbach, who had received his PhD in organic chemistry in 1931 from Krakow's Jagiellonian University. Sternbach went on to do postgraduate research in Zurich, then in 1940 was hired by Roche in Basel. In 1940, the Swiss were highly fearful

that the Germans were about to invade them. Roche decided to send its Jewish scientists to safety, and in 1941 Sternbach was transferred to the Nutley branch plant.

When Sternbach received this directive from on high in 1954, he thought about a class of dyes that he had worked with in Krakow in the mid-1930s. He set to work synthesizing a series of new dye-type products (benzheptoxdiazines) but got nowhere. All were inert in pharmacological testing on animals. Finally, the Roche management asked him to discontinue the research. In April 1957, as he was clearing his cluttered lab bench, he noticed that one of the compounds he had synthesized in 1955 had developed crystals at the bottom. With typical Central-European thoroughness, he decided to send it for testing, promising management that "this would be his last product of this series."[126]

A few days later Sternbach got a phone call from Lowell Randall, Roche's director of pharmacology. This last compound in the series, later called chlordiazepoxide (Librium), had proven very interesting indeed. They were particularly impressed with its "taming effect" in a colony of imputedly vicious monkeys, at doses that did not otherwise affect the monkeys' alertness. Also, "Librium causes a mouse to hang limply when held by one ear; but [unlike meprobamate] it is able to walk when prodded." [127] The drug did seem to have extraordinary qualities. Roche applied for a patent in May 1958.

In January 1959, Roche's medical director persuaded a few psychiatrists to try chlordiazepoxide on some of their office-practice patients. The patients did very well, becoming much less anxious and tense, and sleeping better.[128] Emboldened by the enthusiasm of the psychiatrists, Roche marketed chlordiazepoxide in February 1960 under the trade name Librium. It was the first of the benzodiazepines, or "benzos," and during the 1960s was the number one prescription drug in the United States. Ultimately, there would be more than a thousand kinds of benzos on world markets.[129]

Yet Librium had a number of side effects and could cause fits if suddenly discontinued.[130] Roche felt the series Sternbach was working on Librium as number one on the American drug list.[131] By 1970, one woman in five and one man in thirteen was using "minor tranquilizers and sedatives," meaning mainly the benzos.[132]

The benzodiazepines had a dramatic impact on the practice of psychiatry.

For the first time, psychiatrists were able to offer their patients a potent drug, unlike the mild Miltown, that did not sedate them. (The antipsychotics were simply too potent for routine use in psychiatry.) The share of psychiatric patients receiving prescriptions increased from a quarter of all office visits in 1975 to fully one-half by 1990 (from 25.3 percent to 50.2 percent).[133] With the benzodiazepines as the entering wedge, psychiatry became increasingly a specialty oriented to the provision of medication. With the profession's main previous treatment modality, dynamic psychotherapy, now falling into disuse, an alternative lay at hand.

There was, however, one problem: The benzodiazepines turned out to be addictive, in the sense that patients' symptoms after trying to discontinue the drug were often worse than before starting. In recognition of their potential for abuse, in 1975 the Food and Drug Administration put the benzodiazepines and meprobamate on its "schedule IV," controlling refills and imposing on pharmacists special reporting requirements.[134] Sales had already leveled off, and by 1980 Valium (diazepam) stood number 32 on the list of most commonly prescribed drugs, Librium (chlordiazepoxide) number 59.[135] It was the end of "Valiumania." Nonetheless, the drugs had not exactly gone out of style: almost 7 million prescriptions a year continued to be written in the United States for Valium-style products.

Until this point, there had been very little "unscientific" in the narrative. The benzodiazepines were perfectly appropriate for the treatment of anxiety and mild depression, and science-schooled psychiatrists did well to put their patients on them. Yet it had now become apparent that great sums were to be earned in the sale of psychiatry drugs. As Valium soared in popularity, awareness dawned among drugmakers that here lay the markets of the future. As the highly competitive drug companies rushed into psychopharmaceuticals, they began to distort psychiatry's own diagnostic sense. In trying to create for themselves market niches, drug companies would balloon illness categories. A given disorder might have been scarcely noticed until a drug company claimed to have a remedy for it, after which it became epidemic. As historian of psychopharmacology David Healy puts it, "As often happens in medicine, the availability of a treatment leads to an increase in recognition of the disorder that might benefit from that treatment."[136]

Take, for example, panic disorder. The tradition in psychiatry was to see panic as part of anxiety. As *DSM-II* said in 1968 of "anxiety neurosis," "This

neurosis is characterized by anxious over-concern extending to panic and frequently associated with somatic symptoms."[137] In 1964, however, Donald Klein, then at Hillside Hospital in Glen Oaks, New York, published an article suggesting that panic was really an illness entity distinct from anxiety. Partially funded by the Geigy and the Smith Kline & French drug companies, the study concluded that one could forestall such attacks by staying on medication.[138] As Klein was a member of the *DSM-III* task force, as well as its subcommittee on "anxiety and dissociative disorders," he was able to persuade other members of the correctness of his views. In 1980, with the publication of *DSM-III*, panic disorder became an illness of its own, characterized, it was said, by "the sudden onset of intense apprehension" and marked by physical sensations such as sweating and faintness.[139] The following year, in 1981, the Upjohn Company of Kalamazoo, Michigan, marketed a new kind of benzodiazepine generically named alprazolam (Xanax). Because the market for benzos was sinking at the time, Upjohn attempted to reposition its benzo as a drug specific for the newly created disease entity "panic disorder." In the 1980s, the company funded extensive field trials—orchestrated by Cornell's Gerald Klerman—to establish that panic was really an independent disease for which alprazolam worked wonders.[140] The results were not entirely convincing.[141] Nonetheless, by the early 1990s Xanax had become one of the hottest drugs in psychiatry, prescribed by many psychiatrists in good faith that they were practicing scientifically and that Xanax offered unique hope in the epidemic of panic disorder sweeping the nation. Among insiders, panic jokingly became known as "the Upjohn illness."

Against this background of psychiatric diagnosis increasingly manipulated by pharmaceutical companies arose the psychiatry drug that was to become the household word of the 1990s: Prozac. When Valium came along, both patients and their doctors were willing to define their problems in terms of anxiety once an effective drug existed for treating it. When Prozac, a drug for depression, arrived on the scene, the accent fell on depression as the hallmark of distress. "Our phone rings off the hook every time someone does a story about Prozac," said one physician at Manhattan's Beth Israel Medical Center. "People want to try it. If you tell them they're not depressed they say, 'Sure I am!'"[142]

Prozac's pathway to fame began in July 1953 when John Gaddum, then at Edinburgh and one of the founders of psychopharmacology in Britain,

speculated to a small but influential group of researchers, “It is possible that the 5-HT [serotonin] in our brains plays an essential part in keeping us sane.”[143] That utterance became the “signpost in the sky” of a whole generation of young psychopharmacologists.[144] Gaddum himself had done some of the basic scientific work on assessing serotonin, and the early story was one of those triumphs of British pharmacology.[145] But it followed that if serotonin kept us sane, increasing the availability of serotonin in the brain might counter psychiatric illness.

The scene shifted to the National Institutes of Health in Bethesda, where researchers in Bernard Brodie’s Laboratory of Chemical Pathology discovered in 1957 that an antipsychotic drug named reserpine could unlock the body’s stores of serotonin. The Brodie group correlated behavioral changes with the presence of the various amines, and serotonin became a star. It was the Brodie lab, “LCP,” that opened the whole psychiatric side of serotonin research. “LCP,” recalled one researcher fondly, “was the Camelot of pharmacology.”[146]

But it’s often forgotten that there was a British Camelot as well. The young researchers, inspired by Gaddum, were burrowing into brain chemistry at the same time as the Americans. In 1963, Alec Coppen, a biochemist-psychiatrist of the Medical Research Council and staff member at St. Ebba’s Hospital, did a crucial experiment showing that serotonin-equivalents could relieve depression.[147] Said Coppen later, “I claim this was the first observation that suggested that 5-HT [serotonin] was important in depression—an idea that is now the centre of a multi-billion pound drug market. But for many years, people said yah-boo sucks—there’s nothing in this. Fashions are everything in medicine and 5-HT was not in fashion.”[148]

Coppen well knew. In the late 1960s, Arvid Carlsson had reinforced the news that serotonin seemed to control mood and drive.[149] And Carlsson was coaching a Swedish drug company, Astra Pharmaceuticals, in its efforts to bring to market a drug that would inhibit the reuptake of serotonin in fighting depression. In 1981, Astra brought “Zelmid” (zimelidine) onto the market in several European countries.[150] The experience ended in disaster: two years later zimelidine was withdrawn from use as toxic. Nonetheless, Carlsson and Astra count as the initial pioneers of what would later be called SSRIs: selective serotonin reuptake inhibitors.

One might easily bypass these tales of drug companies and their

misadventures were they not such a key element in late-twentieth-century psychiatry. For at the Eli Lilly Company in Indianapolis as well, SSRIs were coming into fashion in the 1970s. The firm's senior pharmacologist Ray Fuller had been following international developments in serotonin. When Fuller came to Lilly in 1971, he tried to interest the firm in the idea that serotonin might have some action against depression in particular. Lilly was resistant. Recalled Alec Coppen of a conference at Lilly's base in Surrey in the early 1970s, "I'll always remember the Vice President of Research saying 'I thank Dr. Coppen for his contribution but I can tell you we won't be developing fluoxetine [Lilly's serotonin drug] as an antidepressant."'

Yet Fuller, in alliance with Lilly biochemist David Wong, carried the day inside the company. Lilly organized a serotonin-depression team. In the meantime, Lilly had already asked chemist Bryan Molloy to synthesize a series of compounds that might function as antidepressants, minus the side effects of the tricyclics. Wong found that some of these compounds inhibited the reuptake of serotonin at the synapse, thus increasing its availability to the brain. (At present writing, this concept is regarded as simplified: Antidepressants probably do not work by relieving a deficiency of a monoamine such as serotonin.[151]) By 1974, lab tests were in progress on "Lilly 110140,"[152] which shortly received the generic name fluoxetine, later the trade name Prozac. In 1976, Lilly tested a Prozac analogue, nisoxetine, on healthy volunteers, who showed no side effects. The research also established that the drug did not seem to block the reuptake of other neurotransmitters such as noradrenaline,[153] that it too, in other words, seemed to be a SSRI. (The acronym SSRI came into general use only in the early 1990s.[154]) By 1978, Lilly was using the phrase specific serotonin reuptake inhibitor in connection with fluoxetine.[155] Meanwhile, fluoxetine was being put into clinical trials in Indianapolis and Chicago: The results were promising although Lilly—perhaps for competitive reasons-did not publish them.[156]

In 1980, the company decided to go all-out on the drug and sought some big-name biological psychiatrists to help test fluoxetine. John Feighner had left Samuel Guze's department in St. Louis to establish his own private psychiatric clinic in La Mesa, California. Some very good news for the firm flowed from this clinic in 1983: Fluoxetine was just as good as the standard tricyclics in combating depression, and was without huge side effects. There was one more thing: Among the 12 different new antidepressants that Feighner was evaluating, fluoxetine was the only one that had as a "side

effect" weight loss.[157] (The first generation antidepressants often caused weight gains.) For millions of individuals, of course, weight loss is not a side effect but an ardently desired goal. A weight-reduction drug requiring no dietary restrictions would have an enormous market. After Lilly mentioned in its 1985 annual report that the company was developing a drug for weight loss, the stock soared.[158]

Yet the more that Lilly tested fluoxetine as an antidepressant, the more the company put weight loss on the back burner, for the findings that came in from a number of field trials between 1984 and 1987 showed that patients would find fluoxetine much superior to the standard tricyclics because it had fewer side effects, making them feel, if anything, euphoric and "wired" rather than leaden and constipated. It also acted earlier and possessed a safer therapeutic window, meaning a wide margin between the therapeutic dose and the toxic dose (hence patients didn't have to be monitored with blood tests).[159] In December 1987, the Food and Drug Administration approved Prozac for use.[160]

In 1990, three years after Prozac was released, two researchers at McLean Hospital published an article suggesting that the drug was effective not merely for depression but for a range of disorders from panic to drop attacks ("cataplexy"). Since all of these conditions responded to Prozac (as well as to other drugs), they must have something in common, perhaps membership in an Affective Spectrum Disorder (ASD). This created an apparent scientific justification for expanding greatly the notion of depression, which now became one of "the most widespread diseases of mankind," as the authors put it.[161] One of the authors, Harrison Pope, was quoted as saying that ASD affected "possibly one third of the population of the world."[162] The prospects for Prozac became incalculable.

And so the word went out. By 1993, almost half of all visits to American psychiatrists were for mood disorders.[163] Just as Valium had assuaged a nation beset by anxiety, the availability of a new drug for depression had produced a pattern of disorder the drug was capable of treating.

What followed was a media psychocircus of suggestion, as Prozac and its competitors were extended to the world public as a panacea for coping with life's problems even in the absence of psychiatric illness (one recalls that the great majority of individuals with a formal psychiatric illness seek no treatment of any kind). Prozac is "much more than a fad," proclaimed *Time* in

1993. “It is a medical breakthrough” that has brought relief to individuals such as “Susan,” a self-described workaholic who becomes irritable around the time of her periods and once threw her wedding ring at her husband. Now the edges of her personality had been planed off a bit.[164] It would be ludicrous to argue that such people suffered a formal psychiatric illness in the historic tradition of the agonized and the inconsolable, for real psychiatric disease causes terrible pain and disablement. Yet here lay part of Prozac’s core market.

Driven by the promise of problem-free personality and weight loss, Prozac took off more rapidly than any other psychiatric drug in history. By 1990, less than three years after its appearance, it had become the number one drug prescribed by psychiatrists.[165] “With Millions Taking Prozac, A Legal Drug Culture Arises,” headlined the *New York Times.*[166] The black market that was developing for the drug seemed hardly necessary, for physicians were prescribing it for everything imaginable. “Prozac has attained the familiarity of Kleenex and the social status of spring water,” said *Newsweek* in 1994. “The drug has shattered old stigmas,” as Americans were said to be “swapping stories about it at dinner parties.” [167] By 1994, Prozac had become the number two best-selling drug in the world, following, perhaps ironically, an ulcer drug named Zantac.[168]

Inserting Prozac into the history of psychiatry, requires untangling good science from scientism. Good science lay behind the discovery of fluoxetine as a much safer and quicker second-generation antidepressant than imipramine and the other tricyclics, whose discovery Kuhn had launched. Scientism lay behind converting a whole host of human difficulties into the depression scale, and making all treatable with a wonder drug. This conversion was possible only because clinical psychiatry had enmeshed itself so massively in the corporate culture of the drug industry. The result was that a scientific discipline such as psychiatry nurtured a popular culture of pharmacological hedonism, as millions of people who otherwise did not have a psychiatric disorder craved the new compound because it lightened the burden of self-consciousness while making it possible for them to stay slim.

Yet the Prozac episode produced one massive benefit for the public good: It helped psychiatric conditions begin to seem acceptable in the eyes of the public, although we are still far from speaking of a complete destigmatization of mental illness. The “insane,” who had transfixed the public view with horror for centuries, had now vanished to be replaced by people suffering

from “stress” for whom help was easily available. Said *Newsweek* in 1990, a sure guide to the public pulse, “As Prozac’s success stories mount, so does the sense that depression and other mental disorders are just that—treatable illnesses, not failings of character.”[169] In the judgment of historian Healy, the pharmacotherapy of mild depression had evidenced itself to be so successful that one might conclude “‘biological depression’ is a mild illness for the most part and that those who end up being hospitalized for the disorder are an unrepresentative minority.”[170]

There were other players in this beginning destigmatization of mental illness. The National Alliance for the Mentally Ill, or NAMI, a Washington-based advocacy group founded in 1979 consisting of parents and friends of those with major mental illnesses, campaigned against such stigmatizing notions as the schizophrenogenic family and insisted that schizophrenia was a biological disease. By 1996, NAMI had 130,000 members and a staff of 38. Were the mentally ill so awful that local communities should ship them away on a bus? In 1985, a National Coalition of Psychiatrists Against Motorcoach Therapy was formed, dedicated to stopping the practice of “procuring one-way bus fares for habitual and undesirable mental health patients” after their release from the hospital.[171] In various ways, therefore, advocacy groups militated to ensure the acceptance of psychiatric illness as just another kind of medical illness and not something outlandish and fearsome.

Nonetheless, if by the end of the twentieth century, “lunacy” ended up seeming less awful, the success was heavily that of psychopharmacology. It was not that people themselves had become more understanding or tolerant, merely that the drug revolution had made it possible to dampen or abolish entirely the symptoms of psychiatric illness, so that individuals with these disorders needed be no more feared than people with a broken arm or a bruise on the head. “After 37 years,” said Pierre Deniker, who had helped initiate the pharmacological revolution, “the face of madness has been completely changed, not only by means of psychopharmacology but also by the development of psychotherapy, sociotherapy and the rehabilitation of the patients in the community.” For Deniker, “the insane or lunatic” had metamorphosed into “an ordinary patient.”[172] Whether one deems drug therapy to be “cosmetic” psychopharmacology or not, this is no small achievement.

Why Psychiatry?

In two hundred years' time, psychiatrists had progressed from being the healers of the therapeutic asylum to serving as gatekeepers for Prozac. Psychiatric illness had passed from a feared sign of bad blood—a genetic curse—to an easily treatable condition not essentially different from any other medical problem, and possessing roughly the same affective valence. Indeed, so much like other medical illnesses had disorders of the mind become that the uncomfortable question arose, who needed psychiatrists?

Physicians themselves began increasingly to avoid the field. The percentage of American medical graduates planning to specialize in psychiatry fell from 3.5 in 1984 to 2.0 in 1994, a decline of almost half. By the mid-1990s fewer than 500 young doctors were entering psychiatry training programs annually.[173]

The crisis of psychiatry at the end of the twentieth century was not dissimilar to that at the beginning, when neurology and internal medicine had threatened to gobble up office-practice psychotherapy and the asylum menaced entombing psychiatry itself in a mausoleum of red brick. The difference was that by the century's end psychiatry possessed a more solid physical basis than the asylum, namely a proprietary knowledge of the use of drugs.

Yet such specialty knowledge could prove a slim reed. Other medical professions with specialized knowledge had gone under as science raced past their wisdom, witness hydrotherapy. Moreover, psychiatry had a long history of losing ground. Every time a psychiatric disorder became medicalized, it disappeared from psychiatry. Thus over the years, psychiatry had lost neurosyphilis to the internists, mental retardation to the pediatricians, and stroke to the neurologists. Now that psychiatry itself had become medicalized, what justification was there for retaining it as a separate specialty? The psychotherapeutic side could easily be hived off to the psychologists and social workers, who were more intensively trained as therapists, and the brain-biological side assigned to the neurologists, who felt more at home looking at CT scans and determining the significance of lesions in the basal ganglia. What was left?[174]

Consider this confrontation. "I'd been feeling tired, rundown, and very weepy," said a patient. "It seemed relentless, and although I rarely go to the

doctor, I felt it wise to get a good physical and checkup. He listened for about 2 minutes (not even looking at me), suddenly said, 'You're depressed,' handed me a prescription, and walked out." The patient was so angry that she slammed his office door and rushed home in tears. "I could have been a bit of furniture for all he cared—and I didn't even get a physical!"[175]

Psychiatry is a specialty that specializes in the doctor-patient relationship. Whether one is trained on the psychotherapeutic side or the neuroscientific, one learns as a budding psychiatrist that giving one's patients time is the essence of the art. Whereas the average consultation in internal medicine or obstetrics lasts only around 10 minutes, the average in psychiatry lasts over 40.[176]

Within this 40 minutes, psychiatrists do essentially two things that their competitors on either side—the psychologists on the one side, the neurologists on the other—do not do. Psychiatrists offer psychotherapy, which the neurologists generally speaking do not (the average consultation in neurology lasts only 28 minutes, and goes on that long only because the neurological exam is so time-consuming[177]). And psychiatrists prescribe medications, which the nonmedical competition is not permitted to do.

This combination of psychotherapy plus medication represents the most effective of all approaches in dealing with disorders of the brain and mind. Surveys comparing the effectiveness of psychotherapy alone, medication alone, and the two together, converge on the finding that "neurochem" and "neurochat" augment each other as the optimum form of care. "The advantage for combined treatment is striking," concluded one study. "A combination of treatments may represent more than an additive effect of two treatments." Instead, the one seems to potentiate the other.[178] There may be a synergistic relationship between biological improvement from medication and an opportunity to discuss with a sympathetic physician the kind of cognitive disorientation that occurs in psychiatric illness.

The accent here is on physician. Although the skills of the psychologist and social worker are not to be denigrated, the history of medicine suggests that patients derive some kind of bonus from the knowledge that they are dealing with a physician. It seems to be true that the kind of catharsis achieved from telling one's story to a figure of respect is heightened when that figure is not merely a friend or confidant, but a doctor.[179] "Human suffering responds to the spoken word rendered by compassionate persons cast in the role of healer," said one observer. "Even though men have known

this for a long, long time, is it still very good news."[180]